

Cascade Primary Care

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PATIENT

Rivera, Alex
DOB: 08/12/1995 (age 28) · she/her
MRN: CPC-2024-01193

VISIT

Date: June 20, 2024
Provider: Dr. Michael Torres, MD — Internal Medicine
Type: Annual Wellness + Post-Orchiectomy Follow-Up (3 months post-op)

& ALLERGIES: Penicillin (rash/urticaria) · Sulfonamides (hives, facial swelling)

CURRENT MEDICATIONS

- Estradiol valerate 6 mg sublingual daily (per Dr. Chen, gender-affirming HRT)
 - Vitamin D3 2,000 IU daily (started Sep 2023 for deficiency)
 - Atorvastatin 10 mg daily (started Jan 2024 for hypercholesterolemia)
 - Spironolactone 100 mg daily — DISCONTINUED March 2024 (post-orchietomy, no longer needed)
- Surgical History: Bilateral orchiectomy 03/15/2024 (Dr. K. Pham, Swedish Medical Center). Uncomplicated recovery.
Anatomical Note for Chart: Intact prostate. Testes removed (orchietomy 03/2024). No uterus, ovaries, or cervix.

SUBJECTIVE

Alex presents for annual wellness exam and 3-month post-orchietomy follow-up. She reports excellent recovery from surgery with minimal post-operative pain and full return to activity within 3 weeks. Per communications with her endocrinologist Dr. Sarah Chen (Seattle Gender Wellness Center), spironolactone was discontinued post-operatively; estradiol valerate was maintained and dose consolidated to 6 mg once daily.

She was started on atorvastatin 10 mg in January 2024 for borderline elevated cholesterol and has been tolerant of it. She reports occasional smoking (approximately 3 cigarettes per week) and is interested in cessation. She denies palpitations, dyspnea, leg swelling, severe headaches, or vision changes. She reports significant improvement in mood and quality of life since orchietomy.

OBJECTIVE — VITAL SIGNS

BP: 128/82 mmHg HR: 78 bpm Weight: 160 lbs BMI: 24.3

OBJECTIVE — TODAY'S LABORATORY RESULTS

TEST	RESULT	UNIT	FLAG
Estradiol (E2)	245	pg/mL	' in target
Testosterone, Total	12	ng/dL	' excellent
Prolactin	15.8	ng/mL	!' elevated, monitor
Hemoglobin	12.8	g/dL	' appropriate
Potassium	4.3	mEq/L	' normalized
Vitamin D, 25-OH	38	ng/mL	' now normal
Cholesterol, Total	178	mg/dL	' improved
LDL Cholesterol	98	mg/dL	' at goal
HDL Cholesterol	52	mg/dL	normal
ALT	31	U/L	normal
Creatinine	0.82	mg/dL	normal

ASSESSMENT & PLAN

1. Post-Orchiectomy Hormone Management — Excellent Control
Estradiol 245 pg/mL — within HRT target (100–300 pg/mL). No dose changes needed.
Testosterone 12 ng/dL — fully suppressed, as expected after orchietomy. Excellent result.

Continue Estradiol valerate 6 mg sublingual daily. Next hormone labs in 6 months.

2. Prolactin — Mildly Elevated, Monitoring Required

Prolactin 15.8 ng/mL — within normal range (<29 ng/mL) but trending upward from 7.2 baseline. Estradiol use can mildly stimulate prolactin production. This is a pharmacologic effect to monitor. If prolactin exceeds 25 ng/mL on next lab, will order MRI pituitary to rule out adenoma. No headaches or visual changes reported. Low immediate concern; annual monitoring sufficient.

3. Prostate Health — Screening Plan

Intact prostate per surgical record. PSA not appropriate at this time given testosterone <20 ng/dL. Will begin PSA baseline at age 40 or earlier if clinically indicated. No urinary symptoms. No rectal exam today per patient preference. Note: This screening applies regardless of gender identity — patient counseled on importance.

4. Cardiovascular Risk / Cholesterol — Improved on Statin

Total cholesterol 178 mg/dL (down from 215 mg/dL in January). LDL 98 mg/dL — at goal. Continue atorvastatin 10 mg daily. Next lipid panel in 12 months. BP 128/82 — borderline elevated. Lifestyle: reduce sodium, 150 min/week moderate aerobic activity. Smoking cessation counseled strongly. Provided nicotine patch starter kit. Tobacco use + estrogen increases clot risk. NOTE: Elevated cholesterol in this patient is an independent cardiovascular risk factor, not related to HRT.

5. Vitamin D — Now Normal, Continue Supplementation

25-OH Vitamin D 38 ng/mL — now in normal range (30–100 ng/mL). Up from 22 ng/mL at diagnosis. Continue Vitamin D3 2,000 IU daily. Target 40–60 ng/mL for optimal bone health. Bone density screening recommended at age 40 given long-term estrogen use.

6. Preventive Screening — Due and Upcoming

Breast: Clinical breast exam performed today — normal. Mammography eligibility begins at age 40 (2035). Recommend annual clinical exam.
Cervical: N/A — no cervix.
Colorectal: Standard screening begins at age 45 (2040).
STI: Offered full STI panel — patient declined, reports monogamous relationship.

Follow-up: 12 months wellness; 3 months if BP remains elevated or new symptoms

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Electronically signed by: Dr. Michael Torres, MD

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